

Nebido® 1000 mg/4 ml
Solution for injection



1. NAME OF THE MEDICINAL PRODUCT

Nebido 1000 mg /4ml solution for injection

2. QUALITATIVE AND QUANTITATIVE COMPOSITION

Each ampoule/vial contains 1000 mg testosterone undecanoate (corresponding to 631.5 mg testosterone) in a 4-ml solution for injection (250 mg testosterone undecanoate/ ml).
For the full list of excipient(s) see section 'List of excipients'.

3. PHARMACEUTICAL FORM

Solution for injection
Clear, colorless to yellowish oily-brown solution.

4. CLINICAL PARTICULARS

4.1 Indication(s)

Testosterone replacement therapy for male hypogonadism when testosterone deficiency has been confirmed by clinical features and biochemical tests (see 4.4 Special warnings and precautions for use).

4.2 Dosage and method of administration

Method of administration

Solution for injection

Dosage regimen

Nebido (1 ampoule/vial corresponding to 1000 mg testosterone undecanoate) is injected every 10 to 14 weeks. Injections with this frequency are capable of maintaining sufficient testosterone levels and do not lead to accumulation.

The injections must be administered very slowly. Nebido is strictly for intramuscular injection. Special care must be given to avoid intravascular injection. See section 'Instructions for use/ handling' to avoid injury when opening.

► Start of treatment

Serum testosterone levels should be measured before start of treatment. The first injection interval may be reduced to a minimum of 6 weeks. With this loading dose, steady-state levels will be reached quickly.

► Individualization of treatment

It is advisable to measure testosterone serum levels occasionally at the end of an injection interval. Serum levels below normal range would indicate the need for a shorter injection interval. In case of high serum levels an extension of the injection interval may be considered. The injection interval should remain within the recommended range of 10 to 14 weeks.

Additional information on special populations!

Pediatric patients

Nebido is not indicated for use in children and adolescents and it has not been clinically evaluated in males under 18 years of age (see section 'Special warnings and precautions for use').

Geriatric patients

Limited data do not suggest the need for a dosage adjustment in elderly patients (see section 'Special warnings and precautions for use').

Patients with hepatic impairment

No formal studies have been performed in patients with hepatic impairment. The use of Nebido is contraindicated in men with past or present liver tumors (see section 'Contraindications').

Patients with renal impairment

No formal studies have been performed in patients with renal impairment.

4.3 Contraindications

Androgen-dependent carcinoma of the prostate or of the male mammary gland
Hypercalcemia accompanying malignant tumors
Past or present liver tumors
Hypersensitivity to the active substance or to any of the excipients.
The use of Nebido in women is contraindicated.

4.4 Special warnings and precautions for use

Older patients treated with androgens may be at an increased risk for the development of prostatic hyperplasia. Although there are no clear indications that androgens actually generate prostatic carcinoma, these can enhance the growth of any existing prostatic carcinoma. Therefore carcinoma of the prostate has to be excluded before starting therapy with testosterone preparations.
As a precaution, regular examinations of the prostate are recommended in men.

Hemoglobin and hematocrit should be checked periodically in patients on long-term androgen therapy to detect cases of polycythemia (see section 'Undesirable effects').

As a general rule, the risk of bleeding from using intramuscular injections in patients with acquired or inherited bleeding disorders always has to be taken into account. Testosterone and derivatives have been reported to increase the activity of coumarin derived oral anticoagulants (see also section 'Interaction with other medicinal products and other forms of interaction').



Testosterone should be used with caution in patients with thrombophilia, as there have been post-marketing studies and reports of thrombotic events in these patients during testosterone therapy.

Cases of benign and malignant liver tumors have been reported in users of hormonal substances, such as androgen compounds. If severe upper abdominal complaints, liver enlargement or signs of intra-abdominal hemorrhage occur in men using Nebido, a liver tumor should be included in the differential-diagnostic considerations.

Caution should be exercised in patients predisposed to edema, e.g. in case of severe cardiac, hepatic, or renal insufficiency or ischemic heart disease, as treatment with androgens may result in increased retention of sodium and water. In case of severe complications characterized by edema with or without congestive heart failure, treatment must be stopped immediately (see section 'Undesirable effects').

Testosterone may cause a rise in blood pressure and Nebido should be used with caution in men with hypertension. Clinical trials with Nebido in children or adolescents under the age of 18 have so far not been conducted.

In children testosterone, besides masculinization, can cause accelerated growth, bone maturation and premature epiphyseal closure, thereby reducing final height. The appearance of common acne has to be expected.

Preexisting sleep apnea may be potentiated.

Androgens are not suitable for enhancing muscular development in healthy individuals or for increasing physical ability. As with all oily solutions, Nebido must be injected strictly intramuscularly and very slowly. Pulmonary microembolism of oily solutions can in rare cases lead to signs and symptoms such as cough, dyspnoea, malaise, hyperhidrosis, chest pain, dizziness, paraesthesia, or syncope. These reactions may occur during or immediately after the injection and are reversible. Treatment is usually supportive, e.g. by administration of supplemental oxygen.

Suspected anaphylactic reactions after Nebido injection have been reported. Androgens may enhance insulin sensitivity. Therefore, the dosage of hypoglycaemic agents may need to be lowered.

Drug Abuse and Dependence

Testosterone has been subject to abuse, typically at doses higher than recommended for the approved indication and in combination with other anabolic androgenic steroids (AAS). Abuse of testosterone and other AAS are seen in adults and adolescents, including athletes and body builders. Testosterone and AAS abuse can lead to serious adverse outcomes particularly cardiovascular and psychiatric adverse events (See Section Adverse Effects/Undesirable Effects).

If testosterone abuse is suspected, check serum testosterone concentrations to ensure they are within therapeutic range. However, testosterone levels may be in the normal or subnormal range in men abusing synthetic testosterone derivatives. Counsel patients concerning the serious adverse reactions associated with abuse of testosterone and AAS. Conversely, consider the possibility of testosterone and AAS abuse in suspected patients who present with serious cardiovascular or psychiatric adverse events.

Continued abuse of testosterone and other AAS may result in dependence and withdrawal symptoms. Individuals taking supratherapeutic doses of testosterone may experience withdrawal symptoms lasting for weeks or months which include depressed mood, major depression, fatigue, craving, restlessness, irritability, anorexia, insomnia, decreased libido and hypogonadotropic hypogonadism.

Drug dependence in individuals using approved doses of testosterone for approved indications has not been documented.

4.5 Interaction with other medicinal products and other forms of interaction

► Drugs that affect testosterone

Barbiturates and other enzyme inducers

Interactions can occur with drugs that induce microsomal enzymes which can result in increased clearance of testosterone.

► Effects of androgens on other drugs

Oxyphenbutazone

Increased oxyphenbutazone serum levels have been reported.



Oral anticoagulants

Testosterone and its derivatives have been reported to increase the activity of coumarin derived oral anticoagulants, possibly requiring dose adjustment.

Independent of this finding, the risk of bleeding from using intramuscular injections in patients with acquired or inherited bleeding disorders always has to be taken into account as a general rule.

4.6 Fertility, pregnancy and lactation

Pregnancy

Not applicable.

Lactation

Not applicable.

Fertility

Testosterone replacement therapy may reversibly reduce spermatogenesis (see sections 'Undesirable effects' and 'Preclinical safety data').

4.7 Effects on ability to drive or use machines

No observed effects.

4.8 Undesirable effects

Summary of the safety profile

Regarding undesirable effects associated with the use of androgens, please also refer to section Special warnings and precautions for use. The most frequently reported undesirable effects during treatment with Nebido are acne and injection site pain.

Table 1 below reports adverse drug reactions (ADRs) by MedDRA system organ classes (MedDRA SOC)* reported with Nebido. The frequencies are based on clinical trial data and defined as Common ($\geq 1/100$ to $< 1/10$) and Uncommon ($\geq 1/1000$ to $< 1/100$). The ADRs were recorded in 6 clinical studies (N=422) and considered at least possibly causally related to Nebido.

Tabulated list of adverse reactions

Table 1: Categorized relative frequency of men with ADRs, by MedDRA SOC – based on pooled data of six clinical trials, N=422 (100.0%)**

System Organ Class	Common	Uncommon
Blood and lymphatic system disorders	Polycythaemia	Hematocrit increased Red blood cell count increased Hemoglobin increased
Immune system disorders		Hypersensitivity
Metabolism and nutrition disorders	Weight increased	Increased appetite Glycosylated haemoglobin increased Hypercholesterolaemia Blood triglycerides increased Blood cholesterol increased
Psychiatric disorders		Depression Emotional disorder Insomnia Restlessness Aggression Irritability
Nervous system		Headache disorders Migraine Tremor
Vascular disorders	Hot flush	Cardiovascular disorder Hypertension Blood pressure increased Dizziness
Respiratory, thoracic and mediastinal disorders		Bronchitis Sinusitis Cough Dyspnoea Snoring Dysphonia
Gastrointestinal disorders		Diarrhea Nausea
Hepatobiliary disorders		Liver function test abnormal Aspartate aminotransferase increased
Skin and subcutaneous tissue disorders		Alopecia Erythema Rash

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Material No.	93046643	NEW	6 x 6 mm
CNO Mat. No.	Everpharma: 60007282	Previous Mat. No.	Size
Customer Mat. No.	DKSH: N/A		Datamatrix: 60007282
Class. No.	50/000/23		BLACK
Product Name	LF NEBIDO 1000MG/4ML SFJ MY EP		
Fort Size	8.5 pt		
Dimension	148 x 594 mm (net label: 148 x 257 mm)	Grain Direction	N/A
		Date/Version	19.11.2025 V2

